

HEALTHCARE RECEIVABLES · REFERENCE

The Cost of Waiting

Aging, early-out, and why the timing of placement decides how much patient A/R a health system ever recovers

Patient accounts receivable behaves differently from almost any other balance a finance team manages: it is owed by an individual whose contact information decays quickly, it competes with every other household expense, and — on the payer side — it sits behind timely-filing deadlines that permanently close. For a health system, the single biggest lever on how much of that balance is ever recovered is not how hard the business office chases; it is how quickly the account is worked. This reference explains why patient A/R is so hard, how steeply recovery falls with age, and the early-out discipline that recovers materially more.

Executive summary

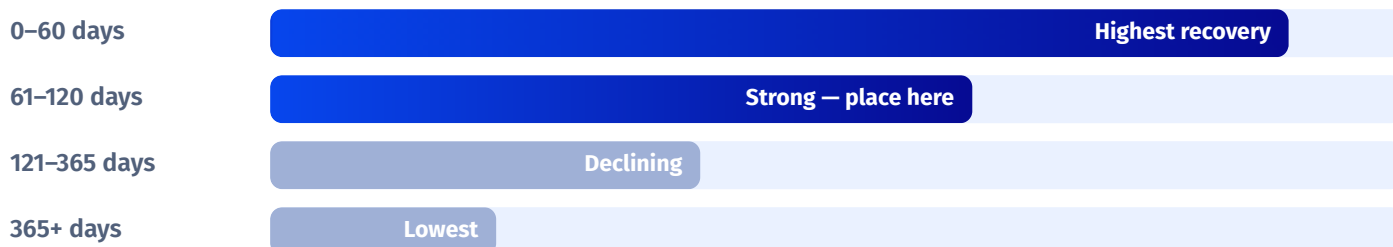
- Patient collections are now the **primary revenue concern for 71% of providers**, and average collection times already exceed **30 days** — before an account ever reaches hard collections.
- Recovery drops sharply once an account passes **90 days**; the guidance is to place with an agency between **90 and 120 days**, before balances cross payer timely-filing limits or drift into write-off.
- Patient financial responsibility keeps rising — the average single-coverage deductible reached **\$1,886 in 2025 (up 43% over ten years)**, with ~34% of covered workers in \$2,000+ deductible plans — so more of every dollar now depends on collecting from the patient.
- On pure contingency there is no upfront cost to placing early, so the decision is purely about timing.

1. Why patient A/R is the hardest balance to collect

Two forces make patient receivables uniquely difficult, and both have been getting worse. The first is the shift of financial responsibility onto patients: as high-deductible plans have spread, the average single-coverage deductible has climbed to \$1,886, a 43% increase over the past decade, and roughly a third of covered workers now sit in plans with deductibles of \$2,000 or more. A growing share of every claim is now the patient's to pay — and patients are far harder to collect from than payers. The second is that the problems that sink a balance are usually created long before it reaches collections: bad eligibility data, incorrect coding, a missing authorization, or a late submission is already embedded in the file by the time the account ages out of the business office. It is little surprise, then, that 71% of providers now name patient collections their primary revenue concern and report average collection times already exceeding 30 days.

2. Recovery falls as the account ages — and the payer clock closes

The age of a patient balance is the strongest predictor of whether it is ever collected. Contact information goes stale, the patient disengages, the balance loses urgency against newer household bills, and — critically — on any portion still tied to a payer, timely-filing deadlines quietly expire, turning a recoverable claim into a permanent write-off. The pattern below is the age curve every patient-AR portfolio follows:



Illustrative and relative — actual recovery varies by payer mix, balance size, documentation quality and contact-data accuracy. Recovery probability drops sharply once an account passes 90 days; Southwest Recovery gives an honest, age-based read on a representative sample before any accounts are worked.

The practical takeaway is specific: place accounts with an agency between 90 and 120 days. Waiting longer routinely pushes balances past payer timely-filing limits or into permanent write-off — value that no amount of later effort recovers.

3. The early-out advantage

The highest-leverage change most revenue-cycle teams can make is to work balances earlier, as an extension of the business office, rather than waiting until they are cold. An early-out program contacts patients while the balance is fresh and the account is still accurate: it validates contact and eligibility data, resolves the small self-pay balances that would otherwise age, sets up workable interest-free payment plans, and offers the online and digital payment options that 62% of consumers now prefer — all before an account ever escalates. Fewer accounts reach hard collections, the cost of recovery falls, and cash comes in weeks sooner. Early-out does not replace bad-debt placement; it runs ahead of it, so the two together capture far more than a single late hand-off.

4. Make it a standing rule

Timing only pays if it is systematic. The revenue-cycle teams that recover the most do not decide account by account whether to place — they run a rule: current self-pay balances go to early-out, and any account still unresolved at 90 to 120 days is placed automatically, wired to the billing system's age triggers so nothing sits. The aged backlog is placed in parallel as a one-time sweep, because on contingency there is no cost to trying and even partial recovery on written-off balances is money already conceded. Measured by placement age, the gap between fresh and aged recovery makes the case for the rule on your own portfolio.

How Southwest Recovery fits your revenue cycle

We work as a compliant extension of your business office, under a Business Associate Agreement and full HIPAA and FDCPA / Regulation F discipline. You place accounts through our secure portal — bulk-uploading a patient-AR / aging export from your billing system — and we map your columns, so there is no re-keying and outreach begins within 24–48 hours. Recovery is omnichannel (phone, email and text with promise-to-pay tracking), built around patient education and flexible, interest-free payment plans, and reported on a live dashboard rather than

for your recovery rate instead of against it, and your staff stay focused on patient care rather than chasing aged balances.

The net effect is a revenue-cycle team that stops losing recoverable dollars to the calendar. Early-out captures the balances that would otherwise age; prompt bad-debt placement captures the rest before payer timely-filing deadlines and write-off rules close the window; and because it is all contingency-based, none of it competes with your operating budget. Timing — the one variable most under your control — finally works in your favor.

About Southwest Recovery Services

Southwest Recovery Services is a HIPAA-compliant medical and healthcare receivables recovery firm, working patient and payer balances on pure contingency since 2004 — for health systems, hospitals, physician groups, and specialty and post-acute providers. We operate under a Business Associate Agreement with every healthcare client, hold staff to ongoing HIPAA and FDCPA / Regulation F training, and furnish qualifying accounts to Equifax and Experian only where lawful (honoring the evolving state medical-debt reporting restrictions). Over two decades we have placed more than \$975M in receivables, recovered over \$76M for clients, and worked 800,000+ accounts nationwide from 12 offices in 7 states — on pure contingency, with no upfront fee, no retainer and no minimum, and a clean compliance record since 2004.

Put a number on your patient A/R

We'll review a representative sample of your patient-responsibility and self-pay balances and give you an honest, age-based read on what is recoverable — and what earlier placement would add.

Call **(866) 551-4684** · sdietz@swrecovery.com · swrecovery.com

Provided for general informational purposes; not legal advice. Statutes and credit-reporting rules for medical debt vary by state and change frequently — confirm current requirements with counsel. Recovery figures are illustrative and vary by portfolio. Southwest Recovery Services is HIPAA-compliant and does not collect consumer debt in California, Oregon or Washington.